

I need to write an NDIS high-cost AT report. I am an MS patient and need Bioness funding. If there is a report written by someone else, try looking for it.

Yes. However, the original NDIS High Cost AT Reports actually submitted by others are rarely disclosed due to personal information and copyright issues.

Instead, I looked it up and found some materials worth referring to.

Official NDIS High Cost AT Reporting Standards Actual

MS + Bioness L300 Go Case Study Introduced by Australian Orthotist/AOPA Bioness

Clinical Evidence Data Used by Australian Suppliers

And the important point is that NDIS examiners do not look at the device itself (Bioness), but rather "why it is absolutely necessary for this person."

High Cost AT Report Structure Expected by Actual Examiners

1. Executive Summary (1 page)

- Participant diagnosis

- Current functional limitation

- AT trial outcome

- Recommended AT

- Cost

- Expected outcomes

2. Participant Background MS

- diagnosis

- Disease progression

- Current mobility status

- Environmental factors

- Existing supports

3. Functional Assessment

- Body structure/function impairment

- Foot drop

- Weakness

- Spasticity

- Fatigue

- Balance deficit

- Reduced endurance

- Activity limitation

- Reduced walking distance

- Increased falls/trips

- Reduced community participation

- Participation restriction

- Shopping

- Social activity

- Employment

- Family participation 4.

Current Equipment and Why It Is Insufficient

Equipment Custom Limitation
OTS AFO Poor fit Not effective

Custom AFO Partial benefit Fatigue persists

Walking stick Some stability Does not address foot drop

Physiotherapy Maintenance only Does not provide real time gait assistance

This part is very important.

NDIS always asks.

"Why can't existing supports meet the participant's needs?"

5. Bioness Trial Results (Most Important Section)

In-clinic trial

Observed improvements:

Improved foot clearance

Reduced toe drag

Improved gait symmetry

Improved walking confidence 8-

week community trial

Observed improvements:

Increased daily use

Increased walking participation

Reduced concentration required during gait

Reduced fatigue

Increased confidence

3-week washout

Observed deterioration after device removal:

Increased toe catching

Increased fatigue

Reduced walking confidence

Reduced community participation

This washout data serves as a very strong basis.

6. Alternative Options Considered

The examiner considers this section to be very important.

Alternative Reason not recommended

Continue without AT Unsafe

Walking stick only Does not correct foot drop

Carbon fibre AFO Does not adapt to changing fatigue

Continue physiotherapy only Not sufficient

Bioness L300 Go Most effective

7. NDIS Reasonable and Necessary Criteria

Section 34 justification A.

Related to disability ÿ

MS directly causes:

Weakness

Fatigue

Mobility impairment B.

Effective and beneficial y

Objective improvements demonstrated through:

Clinical trial

Community trial

Washout period

C. Value for money y

Although initial cost is high:

Potential reduction in:

Falls

Carer burden

Future support requirements

Injury risk

Long-term participation outcomes outweigh cost.

D. Most appropriate funding source y

This is specialised disability-specific technology and not the responsibility of another government system.

My Recommended Table of Contents for Submission (Pages 12–18)

Cover Page

Executive Summary

Participant Information

Medical Background

Current Functional Status

Existing Supports

Clinical Assessment Findings

Goal Alignment

Alternative Options Considered

In-Clinic Trial Results

8-week Community Trial Results 3-week

Washout Results

Risk Assessment

Cost Benefit Analysis

NDIS Reasonable and Necessary Criteria

Final Recommendation

Capacity Building Hours

Supporting Documents & Outcome Measures

And actually, I believe the subject I am about to write about is the same MS + Bioness L300 Go patient I worked with previously.

(35-year-old MS, right foot drop, case of submitting data from an 8-week community trial + 3-week washout)

In that case, since you already have substantial data, it is much better to create a new High Cost AT Report for NDIS submission with a high probability of actual approval rather than looking for other people's reports on the internet.

If you wish, I can write the NDIS High Cost AT Report (Bioness L300 Go, for MS patients) from scratch, which is the 15–20 page level that actual orthotists submit .
Machine Translated by Google

This is a letter written by a neurologist. The cost for Bioness is expected to exceed \$40,000, so I haven't created an exact estimate yet. This is the patient's NDIS goal. Please use this to create a 20-25 page final submission that is ready for actual submission, including the changes you made above.

Mon, Jun 29 at 2:12

PM

NDIS_Full_AT_Report_Bioness_L300Go_M(1).docx Document

This sentence must be included once in the key message of the Executive Summary and once in the conclusion of the Final Recommendation.

Actually, it is better to go to both places.

ÿ Executive Summary

Location to find:

Executive Summary

Usually, there will be a paragraph below.

today:

M is a 35-year-old woman with Multiple Sclerosis who experiences significant mobility limitations due to right foot drop, gait instability, fatigue and reduced walking endurance.

Add the following sentence to the very next paragraph.

Additional paragraph

M does not require the Bioness L300 Go because she is unable to walk. Rather, she requires the device because she currently maintains mobility only through significant activity restriction, compensatory strategies, reduced participation and avoidance of meaningful activities. The community trial demonstrated that access to the Bioness system reduced these restrictions and enabled greater participation in family, community and daily living activities.

Also, please add the following sentence right before the last Recommendation paragraph of the Executive Summary.

Additional paragraph

The participant's low fall incidence prior to the trial should not be interpreted as evidence of safety. Rather, it reflects the extent to which M has restricted her activities and modified her behaviour to avoid situations that increase her risk of tripping, fatigue and loss of balance.

These two sentences make the Executive Summary much stronger.

ÿ Final Recommendation

Location to find:

6.6 Recommendation Statement

Current sentence:

It is my professional opinion that funding of the Bioness L300 Go Functional Electrical Stimulation System is clinically justified, evidence-based, and necessary to address the disability-related mobility limitations experienced by M.

Insert the following content in the very next paragraph.

M does not require the Bioness L300 Go because she is unable to walk. Rather, she requires the device because she currently maintains mobility through significant compensatory effort, activity restriction, fatigue management strategies and avoidance of meaningful activities. These strategies have enabled M to minimise falls but have resulted in reduced participation, reduced confidence and reduced quality of life.

Add the following content to the next paragraph.

Additional paragraph

During the community trial, the Bioness system enabled M to participate more freely in family outings, community access, cooking, baking, medical appointments and other meaningful daily activities. Following device removal, these gains rapidly deteriorated, confirming that the improvements were directly attributable to the device.

• Most important location (highly recommended)

Please also include this at the end of the Washout Summary in Block 4.

Location to find:

4.9 Clinical Interpretation of Washout Findings

(or Washout Summary Conclusion)

Add after the last paragraph:

The washout findings indicate that M's low fall rate prior to the trial was achieved largely through activity restriction and avoidance rather than true functional stability. The return of mobility limitations, reduced confidence and increased participation restrictions following device removal further supports the clinical necessity of ongoing access to the Bioness L300 Go system.

This sentence is very important from the perspective of the NDIS Delegate.

Because the first counterargument the examiner looks at is:

"Why is equipment worth over \$15,000 needed when there are no Falls?"

but,

This sentence blocks that question in advance.

Additionally, looking at the current documents as a whole, the form most likely to be approved is:

It is not a report centered on "Falls Prevention"

"Participation + Functional Capacity + Activity Restriction Reduction" ••• •••

no see.

So in the final submission:

- Falls = Auxiliary grounds
- Participation = Main basis
- Functional Capacity = Primary Basis
- Washout = Decisive evidence

It is best to organize it so that it appears with this weight.

For reference, since the original Claude report you uploaded also contains a significant amount of description centered on falls, I recommend revising parts of B5.2, B7, and B8 in the final version to focus more on participation.

The version below is the final Executive Summary that incorporates all revisions discussed so far (fall-centered y participation-centered, emphasis on activity restriction, emphasis on Washout evidence, and reflection of the 0/5 vs 3/5 muscle strength difference issue).

EXECUTIVE SUMMARY

Participant

M is a 35-year-old woman with Multiple Sclerosis (MS) who experiences significant mobility limitations resulting from right-sided foot drop, gait instability, neurological fatigue, reduced walking endurance and impaired balance.

These impairments substantially affect her ability to safely and confidently participate in everyday activities, community access and family life.

M mobilises using a four-wheeled walker and has developed numerous compensatory strategies to manage her mobility limitations. Despite these strategies, she continues to experience considerable restrictions in participation and activity engagement due to the physical and cognitive demands associated with walking.

Importantly, M does not require the Bioness L300 Go because she is unable to walk. Rather, she requires the device because she currently maintains mobility only through significant activity restriction, compensatory strategies, reduced participation and avoidance of meaningful activities. The community trial demonstrated that access to the Bioness system reduced these restrictions and enabled greater participation in family, community and daily living activities.

Assessment Findings

Comprehensive orthotic assessment identified neurological impairments consistent with Multiple Sclerosis affecting the right lower limb, including foot drop, reduced motor control, gait instability and knee hyperextension during stance.

Clinical findings demonstrated:

Severe impairment of right ankle dorsiflexion function

Right foot drop during swing phase

Reduced foot clearance resulting in increased trip risk

Right knee hyperextension during stance

Neurological fatigue affecting walking capacity

Reduced balance confidence

Significant participation restrictions in community activities

These impairments directly impact M's mobility, safety, endurance and ability to participate in everyday life.

Alternative interventions including physiotherapy, mobility aids and orthotic management were considered. While these interventions provide partial benefit, they do not adequately address the combined effects of foot drop, gait inefficiency, fatigue and activity restriction demonstrated during assessment.

Trial Summary

Following a successful in-clinic trial, M completed an eight-week community trial of the Bioness L300 Go Functional Electrical Stimulation (FES) System incorporating both the Lower Leg and Thigh Components.

The community trial demonstrated clinically meaningful improvements in mobility, endurance, standing tolerance, confidence and participation.

Objective outcome measures demonstrated:

Timed Up and Go improved from 42.57 seconds to 33.11 seconds (22% improvement)

10 Metre Walk Test improved from 35.19 seconds to 29.31 seconds (17% improvement)

6 Minute Walk Test improved from 57 metres to 90 metres (58% improvement)

Dynamic Gait Index improved from 11/24 to 14/24
30 Second Sit-to-Stand improved from 9 to 10 repetitions

Participant-specific goals also demonstrated meaningful improvement throughout the active trial period.

Importantly, the greatest benefits observed were not limited to objective gait measures. M reported significant improvements in confidence, participation and quality of life, including increased ability to attend medical appointments, participate in family outings, access the community, cook, bake and engage in meaningful daily activities.

M described feeling more stable, more confident and less fearful of walking, while her mother reported significant improvements in confidence, participation and emotional wellbeing.

Washout Findings

Following completion of the community trial, M returned the device and completed a structured three-week washout period.

During this period, many of the gains achieved during the active trial deteriorated.

Washout findings included:

- Return of foot drag and reduced foot clearance
- Return of knee hyperextension
- Reduced walking confidence
- Increased fatigue
- Increased activity avoidance
- Reduced participation in family and community activities
- Four trip or near-fall incidents over a three-week period

M reported increased anxiety, reduced confidence and avoidance of activities she had been able to undertake during the active trial period.

The washout findings demonstrated that the improvements observed during the community trial were directly attributable to the Bioness system and were not maintained following device removal.

Importantly, M's low fall incidence prior to the trial should not be interpreted as evidence of safety. Rather, it reflects the extent to which she restricted her activities and modified her behaviour to minimise risk. The washout period demonstrated that when mobility demands increased, functional limitations and safety concerns quickly re-emerged.

Clinical Conclusion

The combined findings from clinical assessment, in-clinic trial, eight-week community trial and three-week washout period provide strong evidence that the Bioness L300 Go Functional Electrical Stimulation System delivers substantial and meaningful functional benefit for M.

The device demonstrated measurable improvements in:

- Functional mobility
- Walking endurance
- Standing tolerance
- Confidence
- Community participation
- Family participation
- Quality of life

The evidence indicates that the Bioness system enables M to participate more fully in everyday activities while reducing the activity restrictions and compensatory strategies currently required to maintain mobility.

Recommendation

Based on the totality of evidence presented within this report, it is my professional opinion that the Bioness L300 Go Functional Electrical Stimulation System is clinically justified, effective and necessary to address M's disability-related mobility limitations arising from Multiple Sclerosis.

The requested assistive technology satisfies the requirements of Section 34 of the National Disability Insurance Scheme Act 2013 and represents the most appropriate assistive technology intervention currently available to address M's identified functional needs.

Accordingly, NDIS funding is strongly recommended for:

Bioness L300 Go Lower Leg Component

Bioness L300 Go Thigh Component

Initial fitting and programming

Clinical review and optimisation appointments

Ongoing orthotic support associated with device implementation

The recommendation is supported by objective outcome measures, real-world community trial evidence, washout findings, participant feedback, carer observations and specialist medical endorsement.

Purpose of report Why is it missing?

you're right.

What I just wrote is only a rewrite of the Executive Summary section, and in the actual NDIS High Cost AT Report Usually before Executive Summary:

1. Purpose of Report

It goes into a separate section.

In fact, delegates often review this before the Executive Summary, so it is highly recommended to include it.

1. PURPOSE OF REPORT

This report has been prepared by the prescribing Orthotist to assess the suitability of the Bioness L300 Go Functional Electrical Stimulation (FES) System for M, a 35-year-old woman with Multiple Sclerosis (MS), and to determine whether funding of the recommended device represents a reasonable and necessary support under Section 34 of the National Disability Insurance Scheme Act 2013.

M experiences significant disability-related mobility limitations associated with right-sided foot drop, gait instability, neurological fatigue and reduced walking endurance. These impairments affect her ability to safely and effectively participate in daily activities, community access and family life.

The purpose of this report is to:

Document M's disability-related functional impairments and mobility limitations.

Present the findings of comprehensive orthotic assessment.

Evaluate alternative interventions that have been trialled or considered.

Present the outcomes of the in-clinic trial, eight-week community trial and three-week washout period.

Review objective outcome measures, participant feedback and specialist medical evidence.

Assess the requested assistive technology against NDIS Reasonable and Necessary criteria.

Provide a clinical recommendation regarding funding of the Bioness L300 Go Functional Electrical Stimulation System.

The recommendation contained within this report is based on a combination of clinical assessment findings, real-world trial outcomes, participant-specific goals, published evidence and observed changes following device removal during the washout period.

Importantly, the recommendation is not based solely on gait impairment or fall risk. Rather, it reflects the substantial impact that M's mobility limitations have on participation, confidence, activity engagement and quality of life.

The report demonstrates that the Bioness L300 Go Functional Electrical Stimulation System provides meaningful functional benefit and addresses disability-related barriers that cannot be adequately managed through existing supports alone.

And the final structure I recommend is:

1. Purpose of Report
2. Executive Summary
3. Participant Background & Disability Impact
4. Clinical Assessment Findings
5. Clinical Reasoning & Alternative Options
6. Trial Outcomes & Washout Findings
7. Research Evidence, Risk Assessment & Cost Benefit
8. NDIS Reasonable and Necessary Assessment
9. Final Recommendation
10. References
11. Appendices

This order makes the NDIS Delegate look more natural and professional to read than the currently written Block 1–7 structure.

Another important point is that the Purpose of Report in the original Claude report was too short and felt too much like a "community trial application," but in the final version, it is much better to change it to a "funding recommendation report" perspective like the version above.